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Department of Pharmacy

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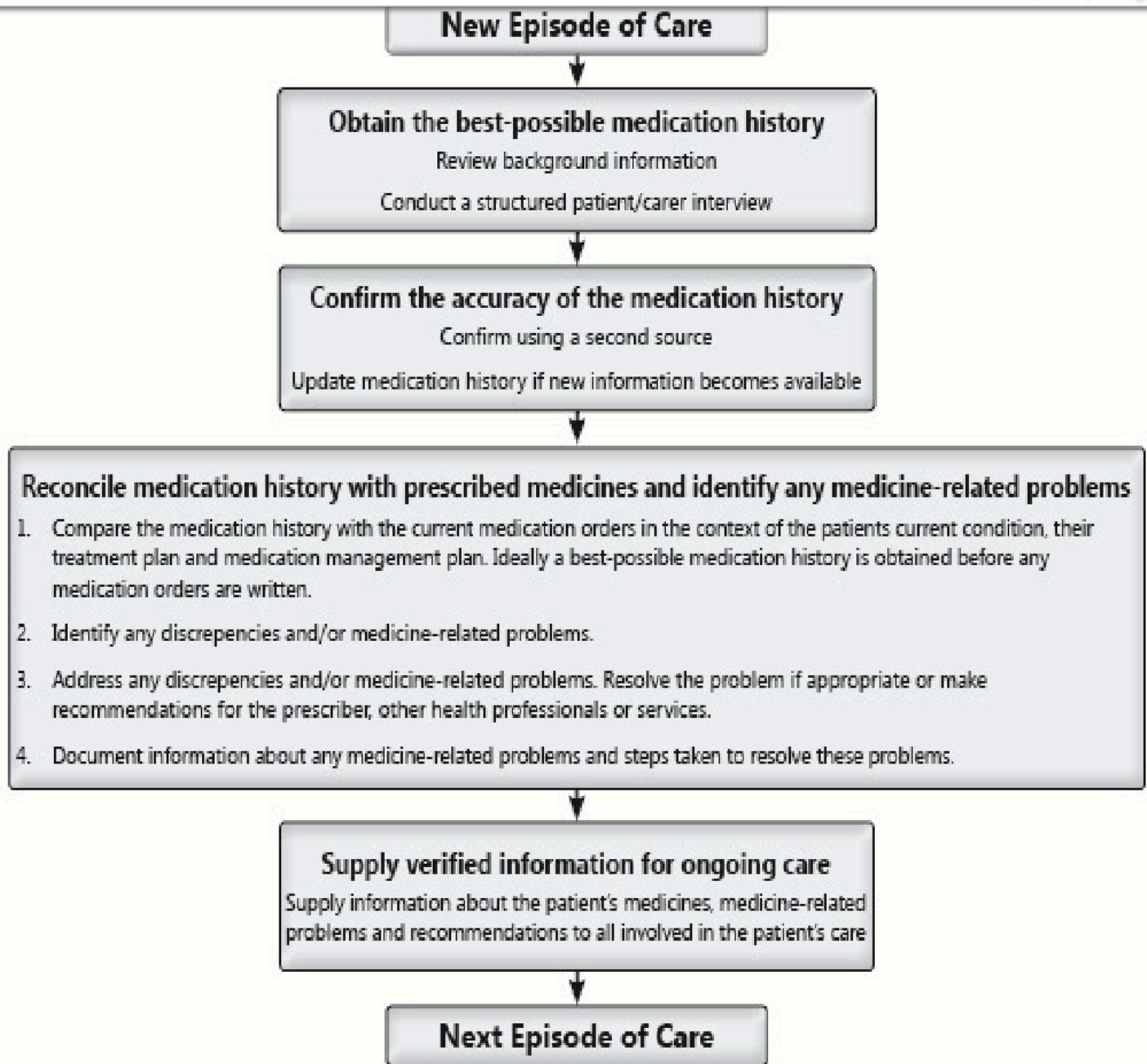
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Title: Medication Reconciliation (Med Rec) Policy

Definition¹

Medication reconciliation (MR) is the standardised process of obtaining a patient's best possible medication history (BPMH) and comparing it to presentation, transfer or discharge medication orders in the context of the patient's medication management plan (MMP).

Figure 1.Medication Reconciliation Pathway¹



Purpose

To demonstrate Royal Hospital commitment to medication reconciliation and its importance as a patient safety initiative.

Statement

- Medication reconciliation is an essential component of safe medication management.
- The process of medication reconciliation is reliant on a team approach.
- Patient/family and/or the participation of an alternate decision maker is an integral component of the medication reconciliation process.
- Initiation of the medication reconciliation process occurs for each patient as determined by a healthcare professional at each of the following care transitions:
 - Admission into the healthcare system
 - Transfer to another healthcare setting
 - Discharge process of exiting the system

Scope

This policy applies at Royal Hospital.

Responsibility

It is the responsibility of each unit/ward to determine specific roles and responsibilities for completing the medication reconciliation process in their area.

It is the professional responsibility of all healthcare professionals that have medication management within their scope of practice and job duties to participate in the collection of an accurate patient medication history and to identify discrepancies in medication therapy.

An **authorized prescriber** is responsible to address and resolve the discrepancies.

Process

The process of medication reconciliation is initiated with the generation of the best possible medication history (BPMH) of all medications the patient is taking at the time of admission.

On admission, medications are reconciled by a health care professional (intentionally continued, discontinued, or modified), where possible.

The healthcare professional shall:

- At admission, generate best possible medication history using at least two sources of information such as but not limited to:
 - o Interview with patient/carer
 - o External electronic medical record (e.g. health Centre, secondary care)
 - o Brought-in-medication packages and/or
 - o Previous admission records/discharge summary/out-patient Rx
- Enter all medication information in the electronic medication reconciliation record where you can the reconciliation alert in the top of the e-prescription.
- Always refer to medication reconciliation records to compare during all care transitions including referral and discharge
- Identify and resolve any discrepancies
- At all care transitions document and communicate to the patient and/or the next healthcare professional:
 - o Any changes to medications that have been made or new medications that have been added, and
 - o The reason for the changes in medications

There should mechanisms established to support, measure, and continuously improve the effectiveness of the medication reconciliation process.

References

Title of book/ journal/ articles/ Website	Author	Year of publication	Page	References:
Alberta Health Services Governance Documents: Medication Reconciliation	AHS	2012		1. George Taylor, et. al. Standards of Practice for Clinical Pharmacy Services. Journal of Pharmacy <i>Pharm Pract Res</i> 43, No. 2 (suppl), 2013.
Accreditation Canada Required Organizational Practice, Medication Reconciliation	ACI			2. Alberta Health Services Governance Documents: Medication Reconciliation. AHS; 2012

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